TRAUMA TREATMENT & SUPPORT OPTIONS

Participant resource • Spiritual Trauma curriculum

A list of options is not a prescription. The right next step depends on what you want help with, your age and health, current symptoms, readiness, preferences, access, and a qualified clinician's assessment.

Highlighting in the presentation means a service may be available locally. It does not mean the approach is better or right for every person.

Established adult PTSD psychotherapies

Approach	What it generally involves
Cognitive Processing Therapy (CPT)	A structured therapy that examines trauma-related beliefs and "stuck points" that keep distress going.
Prolonged Exposure (PE)	A structured therapy that gradually helps a person approach safe reminders and trauma memories they have been avoiding.
EMDR	A structured psychotherapy that includes attention to traumatic memories while using bilateral stimulation. EMDR has strong evidence for PTSD; researchers continue to study how its components work.

Population-specific or distinct protocols

Approach	Important distinction
Trauma-Focused CBT (TF-CBT)	Designed for children and adolescents and usually includes a nonoffending caregiver.
Narrative Exposure Therapy (NET)	A trauma-focused protocol that uses a chronological life narrative. It is different from general Narrative Therapy.
DBT-PTSD / DBT-PE	Specialized protocols that combine DBT elements with trauma-focused treatment. Standard DBT alone is not the same treatment.

Broader or emerging therapies

Approach	Evidence-calibrated description
Somatic Experiencing	A body-oriented approach that uses awareness of physical sensations and regulation. Research is promising but less established than first-line PTSD treatments.
Internal Family Systems (IFS)	A parts-based therapy model. Some people find it useful; evidence for PTSD remains emerging.
Narrative Therapy	A broader therapy that can support identity, meaning, and re-authoring. It should not be confused with NET.
Dialectical Behavior Therapy (DBT)	Can build mindfulness, distress-tolerance, emotion-regulation, and interpersonal skills. Standard DBT is not a first-line PTSD treatment by itself.
AEDP	An experiential and attachment-focused psychotherapy. Evidence for trauma-related problems is emerging.

Adjunctive approaches

Approach	Possible role
Trauma-informed yoga	May support body awareness, choice, regulation, and well-being. It does not replace indicated PTSD psychotherapy.
Expressive arts	May support expression and engagement as an adjunctive approach. The facility does not currently offer an art-specific group or individual art session.
Equine-Assisted Services	An adjunctive service currently available at the facility through a weekly group and individual sessions with the equine facilitator. It is not equivalent to a guideline-supported first-line PTSD psychotherapy.

Framework participants may encounter

Polyvagal Theory offers one way of talking about autonomic states and safety. It remains debated and should not be presented as a psychotherapy, diagnosis, or settled biological explanation. Useful grounding and regulation skills do not require accepting the theory.

Investigational approaches

Psychedelic-assisted approaches combine a drug with structured clinical support in research or regulated settings. As of September 2026, no psychedelic drug has FDA approval for PTSD. Independent use is not the same as treatment and can carry medical, psychiatric, legal, and recovery risks, especially in an SUD treatment setting.

Questions to ask a clinician

- What problem or diagnosis is this approach designed to treat?
- What evidence applies to people like me?
- What training does the clinician have in this specific method?
- What are the risks, alternatives, cost, time commitment, and signs that it is helping?
- How will this fit with my SUD recovery plan, medication, medical needs, culture, and values?
- What happens if I want to pause, change, or stop?

A final permission slip

You can keep faith, redefine it, find a different community, remain uncertain, or leave religion or spirituality. You can have Jesus and therapy, medication, grief, emotions, questions, boundaries, science, or the 12 Steps, too. Your choices belong to you.

This handout provides education, not diagnosis or individual medical advice. In an immediate crisis or danger, contact program staff or emergency services.